0:00

0 seconds

All right, I'm going to try to finish this um section up real quick for you guys.

0:05

5 seconds

Um there's something else that I wanted to mention about aspiration precautions.

0:11

11 seconds

Um so, oh, it says it on here. Okay, never

0:16

16 seconds

mind. I didn't say it in class, but um some smart cookie asked me about it

0:24

24 seconds

afterwards and I was like, "Oh, yes, I did not say it." But that is absolutely true. Um, not only do we want the

0:32

32 seconds

patient to sit upright for all meals and liquids, but we also want to maintain that head of bed up 30 to 60 minutes after eating. Okay.

0:42

42 seconds

Um, yeah. I mean, I know that I told you guys 45°

0:48

48 seconds

45 to 90 degrees for these patients when they eat. I mean, ideally, as I said, you know, like

0:57

57 seconds

Okay. Um, I don't ever eat at 45 degrees, but like a lot of textbooks will say 45 to 90. You guys, when they're and when your patient can sit 90

1:06

1 minute, 6 seconds

degrees, set them up 90 degrees. It is way better for them than to sit 45. It's better for me to sit 90° and 45. Um, so

1:16

1 minute, 16 seconds

90° should be like the gold standard, but maintain that position for 30 to 60 minutes after eating just because they

1:23

1 minute, 23 seconds

are at extremely high risk for aspiration. and they already have dysphasia. We do not want any regurgitation afterwards after they eat.

1:33

1 minute, 33 seconds

Um, so we want to keep them upright. All right, here is the practice question that we didn't get to, but

1:41

1 minute, 41 seconds

nurse caring for a patient with a dysphasia diet and is feeding her pured chicken. Suddenly, the patient begins to choke. What's the priority nursing intervention? And at this point, if you

1:49

1 minute, 49 seconds

would like to hit pause and you figure it out yourself, that's great. But I'm going to walk you through these rationes um and each of these options. So

1:57

1 minute, 57 seconds

I will tell you that potentially all of these options are correct. Okay. Now put

2:04

2 minutes, 4 seconds

oxygen on at 2 L nasal canula. It doesn't really say anything about her oxygen saturation. So would I go right

2:11

2 minutes, 11 seconds

to that one? No. Okay. when it asks you like in a situation like this priority

2:21

2 minutes, 21 seconds

nursing intervention like yes there is a priority but sometimes if they are asking you

2:28

2 minutes, 28 seconds

about a priority intervention they're really just asking you like a sequence of events really what would you

2:36

2 minutes, 36 seconds

do first okay even though it has this word priority I just don't want you guys to get like tripped up by that okay

2:43

2 minutes, 43 seconds

so when this question is prime prime example of that. What is a priority nursing

intervention? It's like, okay, well, what would I in if this was a real

2:51

2 minutes, 51 seconds

life situation, what would I be doing first for this patient? Okay, turn her on her side. I mean, like she's not

2:59

2 minutes, 59 seconds

seizing. Um, but maybe she's like vomiting or something. I mean, I don't know. Like, if if she were to vomit, then maybe I would turn her on her side.

3:09

3 minutes, 9 seconds

if she had stuff coming out of her mouth and her throat, like yeah, I would suction, but it says she's just choking.

3:15

3 minutes, 15 seconds

So, like the very first thing that I would do in the sequence of events is stop feeding her. Like I'm not going to feed her anything else when she begins to choke. And I know that's like a very

3:24

3 minutes, 24 seconds

like well duh. But sometimes like these questions like don't read too much into it. Like that's all they're asking you. What's a priority nursing intervention?

3:32

3 minutes, 32 seconds

You're just going to not feed her anymore? I mean obviously there's more to it. She's

choking. But the first thing we're going to do is to not feed her. Okay.

3:44

3 minutes, 44 seconds

[clears throat] All right. So, let's I have got plenty more questions in here.

3:47

3 minutes, 47 seconds

[sighs and gasps]

3:48

3 minutes, 48 seconds

Let's talk about interal versus parental nutrition. Okay. I'm going to show you some pictures first before we talk about

3:56

3 minutes, 56 seconds

interal nutrition. Um, interial nutrition is when we are feeding the patient and the

4:04

4 minutes, 4 seconds

patient's getting some nutrition via uh their GI tract, but

4:14

4 minutes, 14 seconds

um through a tube. So, they're not eating, they're not on like an oral diet like how you and I eat. they have some

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4 minutes, 21 seconds

sort of tube that is allowing food to get to their digestive system.

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4 minutes, 27 seconds

So they are very much using their GI system and kind of goes along with the

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4 minutes, 34 seconds

like hey just you know the question I asked you of is it better to rehydrate patients IV or PO it's PO why because we

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4 minutes, 43 seconds

want patients using their GI tract if they can we'll talk about parental nutrition but ential nutrition is using

4:50

4 minutes, 50 seconds

that GI tract it's just these patients can't like use their mouth to get food in but

4:58

4 minutes, 58 seconds

but the rest of their digestive system is just fine. Okay, [snorts] so you guys learned in lab how to insert

5:04

5 minutes, 4 seconds

NG tubes. Okay, there are two main kinds of NG tubes that I do want to point out.

5:12

5 minutes, 12 seconds

The first one is the one that you learned how to insert probably and this is your typical Salem sump NG tube.

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5 minutes, 20 seconds

Okay. Um, a lot of tubes now are going to like this little connector because these are really freaking messy, you guys. This is like an anti-reflux port.

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5 minutes, 31 seconds

Um, and so if you don't like there's a little I don't know why I'm not writing anything on here. Sorry. Okay. There's

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5 minutes, 40 seconds

like this little cap on it. This I don't Yeah, it's a cap. Sure. [snorts] Okay.

5:47

5 minutes, 47 seconds

If you don't like close that off when you insert this tube, um all of the GI contents will just go right out there

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5 minutes, 55 seconds

and all over the patient's bed and then all over you. And it's great and it's a good time

and it's definitely happened to me. Um [clears throat] so these are

6:03

6 minutes, 3 seconds

actually much nicer. Like I said, I think a lot of facilities are going to these. And this um this has a val or

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6 minutes, 11 seconds

well it's kind of built in here. It's the same concept as this except it's not like a different tube. So this you just have to change the direction of this.

6:22

6 minutes, 22 seconds

I guess it's a valve. Sure. Saying like, oh, this is to feeding like I want it to be infusing or sucking things out or

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6 minutes, 31 seconds

something. Or I want it to be closed off and I'm going to clamp it for whatever reason. Okay.

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6 minutes, 39 seconds

So obviously when you're inserting your NG tube, you would want it like closed off so that the GI contents aren't going

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6 minutes, 46 seconds

to come all over you like in this one unless you have your little cap on there. Anyways,

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6 minutes, 53 seconds

[clears throat]

6:54

6 minutes, 54 seconds

um these are super easy to use and super nice.

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6 minutes, 58 seconds

So this is your larger bore larger bore NG tube.

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7 minutes, 5 seconds

And we usually use these NG tubes to suck things out of patients and that's why they need to be a little bit bigger.

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So we will learn about bowel obstructions and patho um in a few weeks. Okay, we're going to talk about

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7 minutes, 22 seconds

bowel obstructions and one of the treatments for bowel obstructions is to put an NG tube down this patient. So it's a nasogastric tube. You insert it

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7 minutes, 30 seconds

through their nose. that goes all the way, this tip goes all the way down into their stomach and it has little holes.

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7 minutes, 37 seconds

Um, let's see if I can blow this up a little bit. Well, you guys have done this. Okay, your tube has these little holes right here and these little holes

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7 minutes, 46 seconds

will sit in the stomach and you turn this NG tube onto suction and you suck the things out of their stomach and we call that decompressing their stomach.

7:54

7 minutes, 54 seconds

Okay, we'll talk about the whole indication for it in small bowel obstructions and whatnot, but these are your larger bore NG tubes that we use to suck things out.

8:04

8 minutes, 4 seconds

Okay, you can use it for feeding. Not super rare, but or it's not super it's not super common. Okay, if your patient

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8 minutes, 13 seconds

needs a larger bore NG tube, it's usually to get things out of their tummy. Now, there's another feeding tube, another NG tube called a Dobhof or

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8 minutes, 22 seconds

just your like typical nutritional feeding tube. And so, this feeding tube is a lot smaller bore. It's a lot

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8 minutes, 30 seconds

smaller in diameter. And we use this guy to strictly feed patients and give meds.

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8 minutes, 39 seconds

Okay? So, again, well, you I'm not going to go too too ind depth with that. Okay?

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8 minutes, 47 seconds

[clears throat]

8:48

8 minutes, 48 seconds

But the whole point of why I'm bringing this up is because just because your patient has an NG tube does not mean

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8 minutes, 54 seconds

that it is functional to give interial nutrition. Okay, your larger bore NG tubes typically are to suck things out.

9:01

9 minutes, 1 second

Your smaller bore like your dohoff are usually well no they are for nutritional purposes. Okay, that's not the only way

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9 minutes, 9 seconds

that we can get interial nutrition in somebody. [sighs] So, let's talk a little bit about like who would benefit from interial

9:18

9 minutes, 18 seconds

nutrition. So, these are patients that can't have anything by mouth, but they

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9 minutes, 24 seconds

have a functioning GI tract. So, stroke patients with severe dysphasia where like not even like a pudding thick

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9 minutes, 32 seconds

consistency or pured foods are safe for them to swallow, they need some form of ental nutrition. And so these patients,

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9 minutes, 40 seconds

I mean in the hospital, we usually put that dohoff down and then we schedule them for a um PEG tube, a perccutaneous

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9 minutes, 48 seconds

endoscopic tube, gastric gastra, gastros,

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9 minutes, 56 seconds

a peg, [gasps] perccutaneous endoscopic ga. It's a G tube insertion. I can tell

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10 minutes, 5 seconds

you that. Okay. uh gastric tube. Why is a I don't know. The G's blanking me.

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10 minutes, 11 seconds

Okay. So, this is when we insert Oh, I'll blow this guy up a little bit here.

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10 minutes, 19 seconds

Okay. A PEG or a G tube is a tube that gets placed in the stomach. Okay. And there is access outside of the stomach.

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10 minutes, 31 seconds

Oh, is this just going to show me? No, it's just going to show me. Oh, hey, there it is. Okay, this guy, this is a G

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10 minutes, 38 seconds

tube. Okay, so perccutaneous through the skin. The

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10 minutes, 46 seconds

other end of this tube sits in the stomach directly. Okay, and this is the tube and we can hook up feedings through here and we can give patients nutrition

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that way. So really, you guys, if if they're needing a G tube, they're probably I mean, the

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11 minutes, 2 seconds

chances that they're going to use their mouth again to eat are um slim. Okay. Um the other indication

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11 minutes, 11 seconds

outside of like severe dysphasia, especially those stroke patients that would need a G ttube, advanced dementia,

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because they like forget to eat. Um like their functionality for eating is so low that they would benefit from interal

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nutrition. There's nothing wrong with their GI tract. It's just they can't physically or cognitively eat. Um,

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11 minutes, 36 seconds

a lot of like esophageal cancers, these patients will also have pegs.

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11 minutes, 42 seconds

Again, functioning GI tract past this esophagus. So, we can definitely still use those these organs. We don't want these organs um to just be sitting in

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11 minutes, 50 seconds

there not being used. Okay, that's the whole point of the digestive system is to get food in and digest. So, [clears throat] those are kind of like

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11 minutes, 58 seconds

the three main patient populations that I see with PEG tubes or G tubes. Um,

12:05

12 minutes, 5 seconds

there's another tube. So, you've got your NG tube.

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12 minutes, 10 seconds

If there's this, if you guys are like really looking at this picture, there's something called a nasodinal tube. And this is when we insert um an NG tube

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12 minutes, 20 seconds

past the stomach and we put it in um the proximal part of their dadum.

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12 minutes, 28 seconds

Um there's also a nasog junal tube.

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12 minutes, 31 seconds

That's kind of insane. I don't think I've ever seen one of those. But I do see J tubes every now and then. So

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12 minutes, 38 seconds

instead of a tube a you know perccutaneous tube going directly into the patient's stomach this is a tube that looks just like this but it goes in

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12 minutes, 46 seconds

the patient's junum. So the second part of your small intestine.

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12 minutes, 52 seconds

Um so those are kind of all the different routes that interal nutrition can go through.

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12 minutes, 58 seconds

Um main thing about these tubes because this is these are tubes being inserted

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13 minutes, 5 seconds

into these patients. Now it is the only tubes that we are inserting as nurses are your nasogastric tubes or your NG

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13 minutes, 14 seconds

tubes. Okay. And before we are putting anything down that tube like we placed it in the patient.

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13 minutes, 23 seconds

They're not coughing. They're not like in respiratory distress like they don't have increased work of breathing.

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They're not teipnic. Their oxygen saturation is normal. They're not tacocartic. Anything like that regardless. Okay, we need to check

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13 minutes, 38 seconds

placement before we put anything down that tube. Okay, of course, the main problem that we see with NG tube

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13 minutes, 46 seconds

insertion is if we get it in the wrong hole. Um, and that means that we put this tube down

the patient's trachea instead of their esophagus.

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13 minutes, 56 seconds

Well, like all those things I just mentioned, those are the things that your patient's going to um do if we did put it down the wrong tube. However, not

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14 minutes, 5 seconds

always because think of like maybe you have a pretty abundant patient and you're putting one of these in. Um,

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14 minutes, 13 seconds

you know, patients after cardiac arrest, we like to put oralogastric tubes in tubes down their mouth and into their

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14 minutes, 21 seconds

stomach or an NG tube. Like, they're not going to be able to, we just paralyze them to put a breathing tube in them.

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14 minutes, 27 seconds

So, they're not going to have that reaction. So regardless, you guys, we need to check placement. And the gold standard way for doing this is through a

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14 minutes, 35 seconds

chest X-ray. I'm sorry, it's not really a chest X-ray.

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14 minutes, 41 seconds

A KUB um is the fancy term for like an abdominal X-ray.

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14 minutes, 46 seconds

Okay. So, we are looking to make sure that that tip of that tube sits in the stomach. And so that we know 100% we

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14 minutes, 54 seconds

have evidence in front of us, an image that that tube is in the right spot. Um

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15 minutes, 1 second

because if it wasn't, then we are at risk for causing lots of patient harm.

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15 minutes, 7 seconds

Um or maybe that tube was sitting like right at the distal part of the esophagus, but it's just kind of rubbing

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15 minutes, 14 seconds

against it. We could create skin um mucosal breakdown. they can bleed. It's just not good. We want the

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15 minutes, 22 seconds

tip of this tube in the stomach um if it's an NG tube. So, we need to check placement. And again, the gold standard

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15 minutes, 29 seconds

way for doing that is a KUB or abdominal X-ray.

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15 minutes, 34 seconds

Um other things, other facilities in your book, we'll talk about it, too. You can do like gastric pH testing.

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15 minutes, 42 seconds

Um they probably taught you when you did NG tube insertion that you're going to like blow a little bit of air through the syringe. um

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15 minutes, 51 seconds

and and oscultate this the left upper quadrant just to make sure that you hear like the gurgle

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15 minutes, 59 seconds

from the air. Um it's not a perfect science. That's a very good indication that that's in the right spot, but we always get that KUB. Okay. So, we just need to check placement.

16:08

16 minutes, 8 seconds

[gasps]

16:09

16 minutes, 9 seconds

Um when we start a patient on interial feeding too, we need to start them very very slowly on it. We don't want to just

16:16

16 minutes, 16 seconds

be like, "Okay, yeah." Like, oh, you know, we keep trying to do a speech a swallow evaluation on Mima

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16 minutes, 25 seconds

for the last six days and she keeps failing her swallow screen, so now we have to put an NG tube in and she hasn't had any nutrition in six days. But we

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16 minutes, 33 seconds

can just start Mima on like heavy duty um like all this fluid at once in their stomach. No. No.

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16 minutes, 41 seconds

um we start very very slowly and we titrate up with the volume that we give patients for nutrition and that's just

16:49

16 minutes, 49 seconds

so that the patient I mean you have a lot of complications here okay it's really to keep the patient comfortable a lot of these interial feeds can cause

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16 minutes, 58 seconds

very very loose stools okay so diarrhea is definitely a complication hypoglycemia absolutely if you have a diabetic patient that needs interal

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17 minutes, 6 seconds

feeds there are special diabetic formulas there's renal formulas is basically like all of our diets that we

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17 minutes, 13 seconds

talked about earlier today, they make some sort of modification in like liquid interm.

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17 minutes, 21 seconds

Um, but the main one for those patients that haven't had nutrition in a while

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17 minutes, 27 seconds

and we like just slam them with a bunch of [laughter] nutrition at once. Well,

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17 minutes, 35 seconds

first they're going to be like super uncomfortable and they're going to be bloated and probably have a lot of diarrhea, but also we're very at risk for something called refeeding syndrome.

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17 minutes, 45 seconds

And refeeding syndrome occurs um after rapid introduction of interal feeds when this patient has been malnourished for a

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17 minutes, 54 seconds

very long time. Okay. And what happens is

18:03

18 minutes, 3 seconds

when we our cells have been so deprived of everything for so long that when we finally give it to them our cells get so

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18 minutes, 10 seconds

happy and they're they're like this actually they're like this.

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18 minutes, 15 seconds

Okay. Um when we finally get something they get very excited and they pull everything into the cell. Okay.

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18 minutes, 22 seconds

Actually, insulin helps do insulin helps does this. Insulin helps do this. I don't know. Anyways, English is hard.

18:34

18 minutes, 34 seconds

The main electrolytes that get pulled into your cell are potassium, phosphorus, and magnesium. So, with refeeding syndrome, we're very at high

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18 minutes, 43 seconds

risk for low levels of potassium, phosphorus, and magnesium.

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18 minutes, 49 seconds

So that is a huge reason as to why we start these feedings very very slowly.

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18 minutes, 55 seconds

Um we talked a lot about aspiration precautions with the patient that is just on a regular dysphasia diet. But um

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19 minutes, 3 seconds

remember that regurgitation can also cause aspiration.

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19 minutes, 10 seconds

Aspiration is anything foreign in the lungs that should not be there aka anything but air. So um

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19 minutes, 18 seconds

even your patients on tube feeds they are very high aspiration risk

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19 minutes, 25 seconds

because this sphincter can get pretty floppy and this can regurgitate back up and then slip back down the trachea and

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19 minutes, 34 seconds

into the lungs. Okay. So anyone on a tube feed, we want their head of bed at least 30 degrees at all times. If we

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19 minutes, 41 seconds

have to lay the patient flat or you know, turn them, whatever. We are

19:49

19 minutes, 49 seconds

holding the feed. We are turning them and then we are putting their head of the bed back up and restarting that feed. Okay, we do not have this tube

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19 minutes, 57 seconds

feed running when the patient is anything lower than 30° with their head. Okay, that is very very important.

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20 minutes, 5 seconds

[sighs]

20:06

20 minutes, 6 seconds

Other thing with interial um feeds is we need to check gastric residual volumes.

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20 minutes, 12 seconds

So your big like number red flags for gastric residual volumes is two times of 250 mls or more

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20 minutes, 22 seconds

or one time 500 ms or more. Okay. So, a gastric

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20 minutes, 28 seconds

residual volume is before I like usually if the patient's on a continuous feed, meaning like they are

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20 minutes, 36 seconds

getting a little bit of nutrition constantly, like just like an IV infusion, like they're always hooked up to the IV and they're just getting this

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20 minutes, 44 seconds

bag of saline. It's the same idea with your interial feeds. They're just hooked up to their feed. Here, I'll show you a picture.

20:54

20 minutes, 54 seconds

Okay. Oh, it's so blurry. These are different like types of feeding um bags.

21:03

21 minutes, 3 seconds

Um I've seen both of these. This is like a bolus feed. So patients that aren't on a continuous feed. This is a continuous feed. Oh my gosh, sorry I'm not using my

21:12

21 minutes, 12 seconds

pen. This is a continuous feed. This is a continuous feed. If it's a continuous feed, it needs to be on a pump um so it can deliver the appropriate amount every

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21 minutes, 20 seconds

hour. And then you have your water flush that can be programmed in there as well.

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21 minutes, 25 seconds

And um your water flushes should be about every 4 to 8 hours,

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21 minutes, 32 seconds

usually four hours. Um give them some water. So those are your continuous. Your bolus

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21 minutes, 40 seconds

feeding um comes in little like cans, cartons like this. And so if it's like, oh, you

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21 minutes, 47 seconds

know, your patient gets fed four times a day, then you just do however many cartons of this four times a day and

21:56

21 minutes, 56 seconds

you know, with water, of course, because we need to keep them still hydrated. Um, but that's a that's a bull versus a

22:03

22 minutes, 3 seconds

continuous. And I lost my train of thought. I don't know what I was talking about. Oh, gastric residual volumes.

22:07

22 minutes, 7 seconds

Okay, so this [clears throat] looks a little bit different for your continuous versus your bolus feeds. Okay, if it's just a bolus feed or anytime If the

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22 minutes, 15 seconds

patient isn't normally continuously hooked up to their feed, their interial nutrition,

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22 minutes, 22 seconds

um, anytime we go to put something in that tube, whether we're going to feed them this bolus

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22 minutes, 28 seconds

of their, you know, feeding, we're going to give them medications, we should be pulling back. We should be hooking up that like syringe.

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22 minutes, 39 seconds

Um, I was going to say a piston, but I think they've moved away from pistons and I think they're all like um, oh my gosh, I don't remember the adapter on

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22 minutes, 47 seconds

them. Anyways, that big old like 60 mil syringe and we will we'll be pulling back on it and seeing what the residual

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22 minutes, 55 seconds

volume in the stomach is. So, that's what I mean by these like these numbers to remember. Okay, it's if if we pull

23:04

23 minutes, 4 seconds

back 250 milliliters or more, sorry, I should say 250 to like

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23 minutes, 13 seconds

500 or 4.99 or whatever. Um, before I give a medication or a feeding

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23 minutes, 22 seconds

and like that's cool. I can watch it, but the next time I go to do it like 4 hours later and if it's still like 300

23:29

23 minutes, 29 seconds

ms, then I need to I need to stop the feed, hold the feed, and contact the physician and be like, "Yo, um, we are

23:38

23 minutes, 38 seconds

like this patient is not um they have way too high of gastric residual volume because ultimately what that means is that they're at higher risk for

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23 minutes, 45 seconds

aspiration and that regurgitating back up, okay? And they're probably going to be very uncomfortable. There's always 300 milliliters of stuff sitting in

23:52

23 minutes, 52 seconds

their stomach. If I pull like 600 mills one time, I'm doing the same thing.

23:58

23 minutes, 58 seconds

Stop. We're holding the feed. They have like gastric motility issues. We need to figure that out. Decrease the rate of

24:06

24 minutes, 6 seconds

the feed. Something. Um, but we're our first priority action is to stop the feed. If we pull more than 500 mls out

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24 minutes, 15 seconds

one time, two times we have a little bit of leeway. If it's between 250 and 500, we watch it again and pull. And the next

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24 minutes, 25 seconds

time if it goes down, that's great. If it's still between 250 and 500, we're stopping that feeding and holding and contacting the physician. Okay.

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24 minutes, 34 seconds

Um, let's see what else.

24:42

24 minutes, 42 seconds

Um if we are giving things through these um tubes, okay, we should be giving them by gravity, especially like your G

24:51

24 minutes, 51 seconds

tubes, your J tubes. Um give them by gravity. Your feedings, yeah, if it's continuous, it needs to be

24:59

24 minutes, 59 seconds

on a pump. Okay. But if we're giving bololis feedings or medications, they need to be um they should be by gravity.

25:09

25 minutes, 9 seconds

Um, just some like med tips for you guys. If you're giving medications through tubes,

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25 minutes, 16 seconds

you want to make sure that you are only using medications that can be crushed.

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25 minutes, 20 seconds

So, no intercoated, no like extended release. Um, you're crushing them up very very

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25 minutes, 28 seconds

finely. You were doing one medication at a time and you were mixing the crushed very fine powder medication with 15 mls

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25 minutes, 37 seconds

of water and you were doing one medication at a time. Okay.

25:42

25 minutes, 42 seconds

Um, you should also be flushing that tube with 30 mls Oh my gosh, sorry. I should write this down. Okay. 30 mls

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25 minutes, 51 seconds

of water before and after meds.

26:00

26 minutes

Um and this there's another complication on here too of tube occlusion. Okay, these tubes are very very easily clogged

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26 minutes, 8 seconds

um like with medications especially if we don't crush them well they've got clogged and

the patient has to get a new one unfortunately. There are some tips

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26 minutes, 15 seconds

and tricks, but me, if it's clogged, it's a good chance it's going to be clogged, okay? And we're not

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26 minutes, 24 seconds

going to like forcefully push against this clog, okay? We're just we're going to sense resistance. We're going to stop

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26 minutes, 32 seconds

[laughter]

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26 minutes, 34 seconds

and we will be contacting our physician and probably be replacing that tube.

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26 minutes, 40 seconds

Okay, so here's some questions. Patients receiving continuous interial feeding at 65 ms an hour. The residual volume in 4 hours was 125. What's the priority

26:47

26 minutes, 47 seconds

nursing intervention? I told you guys to remember red flag at 250* 2 red flag at 500 * 1. Okay, we have

26:56

26 minutes, 56 seconds

125. Is this okay for our patient? Yeah, it's appropriate. So, what's the priority nursing intervention? Well,

27:04

27 minutes, 4 seconds

raise the bed to at least 45. That should already be done. Continue the feedings. This is a normal gastric residual for this feeding. Yes. Hold the feeding until you talk to the physician.

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27 minutes, 12 seconds

No, we don't need to hold that. Okay.

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27 minutes, 15 seconds

Also, raise the head of the bed at least 45. We know it's 30. 30 for interal feeds. Assess bowel sounds. That's not

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27 minutes, 23 seconds

my I mean like sure, but it's this is a specific question about a residual volume. So,

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27 minutes, 33 seconds

this is like there's no red flags here is what I'm trying to get at. If there's no red flags then I can skip assess

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27 minutes, 41 seconds

and I can continue the feeds nurses and um this is an assist of

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27 minutes, 49 seconds

personnel. Okay AP assistive personnel perform the following interventions for a patient receiving continuous internal feedings. Which action would require

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27 minutes, 57 seconds

immediate attention by the nurse? So which of the following um actions are wrong? So, fastening the

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28 minutes, 5 seconds

tube to the gown with new tape. I mean, that's fine. It prevents like a lot of movement. I don't really care about that. Placing the patient supine when giving a bath. Oh, no. Potential.

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28 minutes, 17 seconds

Monitoring the patient's weight as ordered. Yeah, we'd want to do that.

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That's nothing wrong. Ambuling the patient with interial feeding still infusing. I mean, that's fine. As long as they're not laying flat, that's the

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concern. We do not want them laying flat while they have continuous interal feedings.

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28 minutes, 35 seconds

Okay, let's talk about parental nutrition. Okay, parental nutrition is um your IV nutrition. So, this is

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28 minutes, 44 seconds

nutrition going directly into the bloodstream through a vein. And a lot of these

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28 minutes, 51 seconds

um mixtures, these nutritional Yeah. mixtures, I guess,

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28 minutes, 59 seconds

compositions better word, um, are extremely

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like costic to the veins. They're very hard on the veins.

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And so, we want to make sure that we are putting these IV nutrition forms through a large vein.

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So, let me show you some pictures. Okay.

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29 minutes, 26 seconds

If a patient is on parental nutrition because these are very very hard on veins, we do not want to be using a little vein in the hand to be giving

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these um you know things to our patient. Okay, they need to be in a very large vein. So these are different types of central

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lines. Okay, we talked about central lines oh I don't know one of these weeks. Okay.

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Um let me actually show you a picture.

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29 minutes, 54 seconds

Oh, dang it. It doesn't have it, but that's fine. Okay. This is your heart. All right.

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30 minutes, 2 seconds

This is a central line. This is TPN, total parental nutrition.

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30 minutes, 7 seconds

Um, this line is going directly into his chest into a central venus catheter.

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This catheter will travel along the subclavian and go right Oh, sorry, that's a left. go right into the right

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atrium of the heart. Okay, so these are extremely large veins and again we need them because a lot of

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30 minutes, 30 seconds

the stuff in these um forms of nutrition are extremely hard on the veins.

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30 minutes, 38 seconds

Okay, [clears throat] this other one PPN peripheral parental nutrition this is going into a larger vein in the

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30 minutes, 48 seconds

patient's arm via a pick line. Okay. A pickline. That is this picture here. A pickline.

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30 minutes, 56 seconds

This is a um a jugular. [clears throat] This is an an internal jugular central venus catheter. Okay. A big old vein.

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31 minutes, 8 seconds

Pick line. Usually they go in your um syphalic. It's a big old vein. Okay. So,

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31 minutes, 16 seconds

I need big veins. they need um central lines for these forms of nutrition.

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Okay. [sighs] So, why would I choose this patient to have IV nutrition over

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ential nutrition? Well, parental nutrition is reserved for patients that do not have a functioning GI tract. So,

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I see parental nutrition a lot in patients that um have like recent GI surgery.

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Um maybe it's like well no that doesn't even make sense. Um a Crohn's flare up.

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31 minutes, 58 seconds

Um something is wrong with their GI tract. Okay.

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It could be like GI cancer of some sort they're trying to manage for a short period of time. But at the end of the day, parental nutrition is shortterm.

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32 minutes, 13 seconds

Okay. Interal nutrition can be longterm.

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parental nutrition should be short-term because of all the risks involved. Okay.

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[snorts]

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32 minutes, 23 seconds

And as this third bullet says, the goal is to move from parental to entail to oral nutrition. Okay. Sometimes we have

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32 minutes, 31 seconds

to stop with ential nutrition, but parental should never be a long-term fix. No, no, no, no, no, no. Okay.

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Um there is a little bit of difference between TPN that total parental nutrition that has much higher levels of

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sugar content which is going to make it very harsh on the veins and your peripheral parental nutrition or your PPN. These can be given IV

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um just like a regular IV um because they don't have as like

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you know harsh things in them that are going to be super damaging to the vein.

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But you want to make sure if this patient is getting PPN that it's a very good IV and ideally like a 20 gauge IV

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or larger. um and that it is going into a large vessel. Okay.

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Um but your TPN you absolutely need um a central line for.

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Okay.

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33 minutes, 39 seconds

Oh. [clears throat] Um some things to note about parental nutrition. All right. This is what they look like. Okay.

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Um, if it's like a milky kind of substance and that's what it looks like, um, then there are lipids involved,

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okay? Because we can infuse patients with lipids.

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34 minutes

Um, and that gives it like a kind of a milky consistency.

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The thing with lipid infusions, so these are these are TPN, okay? And they are going to go through a central line. And

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there's a couple of like funky things with um lipid infusions.

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So, we add lipids to TPN for patients with like fat deficiencies.

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Um we need to make sure that the patient doesn't have any egg or soybean oil allergies

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because that's like primarily what they're using for these lipids.

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Um if there's like oil droplets in here or there's like some sort of like creamy

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[snorts]

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34 minutes, 48 seconds

layer.

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Okay. Um that is those those are red flags. Um and we should be returning this TPN to the pharmacy and saying I

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34 minutes, 58 seconds

don't like the way that this looks. This doesn't look right. It should be nice and uniform. There shouldn't be any like fat droplets or anything in there. If there's fat droplets and I'm infusing

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fat droplets, this is a very high risk for a fat embolis. Okay?

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And so we don't want to be doing that to our patient. Um the other thing with lipids is

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they have normal TPN, PPN you it's it's used within 24 hours.

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So we're changing the lines like the IV tubing. They're changing the bag every 24 hours. But if there's lipids involved, it's every 12 hours.

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Okay, so lipids are good for much shorter period of time than just regular TPN.

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Um, oh, TPN that does not contain lipids needs to have a micron filter.

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Okay, so we need to make sure that we have an extra protection before it reaches the patient. And it's just a little filter like this is your IV

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35 minutes, 58 seconds

tubing. Okay, great. And this is where you would regularly hook it up to the patient, but instead there's going to be this extra little filter at the end. And

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it's just to make sure that the patient is getting safe filtered nutrition.

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Okay. Lipids are too big to go through this filter. So, we don't use the filter on lipids. But

again, we're checking our bag really, really well to make sure that there's no like globules in there.

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36 minutes, 24 seconds

Um, if you're wondering why this bag looks yellow, um, it's because it has a lot of B vitamins in it. So, we can make

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parental very very specific to the patients needs unlike ental nutrition.

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36 minutes, 37 seconds

Ental nutrition, it's like, oh well, we have like isosaurus, diabetes, osmolite, which one's best? No, I mean, there's

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more options than that, but we can get very, very um specific with our parental nutrition for what the patient needs.

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36 minutes, 53 seconds

Okay, so those are the main ideas with parental versus entail nutrition. And again, like kind of like in patho, like

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37 minutes

if charts work for you, um you can definitely do like a compare and contrast your parental versus your ential nutrition.

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All right. Patients receiving both PN and EN, when would the nurse collaborate with the healthcare provider and request a discontinuation of the parental nutrition?

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37 minutes, 19 seconds

Okay, so we do not like patients on parental nutrition. We like to have them use their GI system. So the answer for

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37 minutes, 27 seconds

this one is D. Okay. When 75% of the patients nutritional needs are met by

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37 minutes, 34 seconds

tube feeding, that means that hey, you know what? I think that that's pretty good and we can get this patient off of

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IV nutrition. Their GI system is working well enough to meet 75% of their needs.

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Okay, we can get them off that um parental nutrition. Um this is way too small of a number.

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Bowel sounds return. That's great if they have bowel sounds, but maybe they have some other issue like Crohn's. Um and they're, you know, inflammation is so bad they're not absorbing anything.

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Um that doesn't really tell us anything specific. And the central line has been in for 10 days. Um there's no time limit

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on these central lines um that these patients have, but we do want to take them out as soon as possible, but there's no like set time frame of like,

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oh, I don't know, it's day 12. It has to go. There's nothing like that. We we need it for as long as it's indicated, but if it's not indicated anymore, then

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yes, let's please remove that central line.

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[sighs] All right. Patients receiving TPN, whether primary interventions a nurse should follow to prevent a central line infection. Okay. So this kind of just talks about clabsies if you guys

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remember. It's not it's still a funies question but it's not like super TPN based but still a good thing to talk about.

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So yes, we want to change the dressing using sterile technique. Okay. Um change the TPN containers every 48 hours. No,

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this is every 24 which is C. And that's great. Monitor glucose levels to watch and assess for glucose intolerance.

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Um, I mean, yeah, we would be doing that, but that's not necessarily to prevent a central infection, and we don't really care about like aspiration

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isn't a risk in TPN. So, your answers should just be A and C for this one. Let me just double check myself. There we

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39 minutes, 21 seconds

go. Okay. Because like, yes, we need to monitor glucose, but again, that doesn't really speak to the central line infection. Okay.

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All right. Food medication interactions.

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There's just a couple more slides on here.

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I'm sure that you guys talk about this in farm too, but these are like your main contra indications.

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Okay, we don't like anything with grapefruit juice. Vitamin K we talked about a few weeks ago in patho. We need

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vitamin K for clotting. And so if we're taking an anti-coagulant like warerin and we take vitamin K then there's we've

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39 minutes, 59 seconds

got like oh I'm trying to clot but I'm not trying to clot like what's going on that's not going to work well. Okay so the main thing with warerin is um or

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couadin is and vitamin K is that we just want the patient to maintain a consistent intake of vitamin K. Okay. If you like

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the leafy greens that's fine but just like be consistent with it. Don't have like a bunch one day and then not any the next three weeks because that's

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going to throw off your couadin. So just be consistent with your vitamin K intake so that we can get you the right dose of your couadin. Tyramine foods and mais

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um these are um medications you'll probably talk about in psych a little bit more or mental health. Um, tyramine foods are

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like aged cheese, red wine, um, cured meats, soy sauce.

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Okay, so these foods will interact with your MAIs and cause life-threatening hypertensive crisis. There's your

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41 minutes, 2 seconds

tetracycans, your fluorocalones or your cypro and dairy.

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Um, and then we talked about salt substitutes with potassium as well. Don't worry too much about these.

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Um, and we talked a lot about this too, okay? Every disease is going to have its own like, you know, anything cardiac

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41 minutes, 24 seconds

should be on a cardiac diet. Diabetes should be on a diabetic diet. CKD, anything wrong with the kidney should probably be on a renal diet. Okay?

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41 minutes, 35 seconds

Um, just think about like the disease process of what's going on and what this patient will

need. We talked about burns. We talked about wounds. Um, GI

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41 minutes, 43 seconds

patients, we talked about, you know, when a high-fiber diet would be indicated, when a low fiber diet would be indicated. If you guys like know what's going on with the patient, then

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the diet should make sense. So, I'm not going to spend like I think that we already talked a lot about this in class. And the last question I have, so

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42 minutes, 1 second

which action can a nurse delegate to the assisted personnel? Okay, so these are like your CNAs.

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Um, they cannot teach, so stop there.

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42 minutes, 12 seconds

They cannot administer and they cannot evaluate, but they can do tasks. Okay, so they can check a blood sugar and that is fine.